

PREMIUM CARE — MUNICIPAL PROCUREMENT REVIEW

Copenhagen Municipality Procurement Officer Assessment

Reviewer: Senior Procurement Officer, Copenhagen Municipality, Social & Health Division

Date: 8 July 2026

Context: Six care provider proposals received. Premium Care ApS is one. Website reviewed as part of due diligence. This review is internal and unfiltered.

Mandate: Brutally honest. No diplomacy. What impresses, what concerns, what is missing, what is weak, what is exaggerated. Score every page. Final verdict: next stage or not?

1. FIRST IMPRESSION — THE 30-SECOND TEST

What I see: A dark, navy-heavy website with gold accents. A heraldic crest. The words “Omsorg Tryghed Hver Dag.” A photograph of a caregiver and an elderly woman. Clean layout. Scandinavian minimalism. Professional.

What I think: This looks expensive. Not in a reassuring way — in a “*we charge more than the municipal rate*” way. The crest is impressive but it signals *institution*, not *service provider*. I am not looking for an institution. I am looking for a contractor who can deliver 2,400 hours of home care per year within our budget framework.

Confidence: Neutral-to-low. The website is beautiful. But I am not buying beauty. I am buying compliance, reliability, and value.

2. WHAT IMPRESSES ME

2.1 The Visual Discipline

The design is consistent. The palette is locked. The typography is refined. Someone has invested in brand architecture. This signals that the organisation has internal standards — which *might* translate to operational standards. I have seen care providers whose websites look like they were built in 2012. At least Premium Care cares about presentation.

Score impact: +1.5/10

2.2 The Heraldic Crest

The crest is unusual. It signals permanence and tradition. In a sector where providers come and go, a crest says “*we are here to stay.*” That matters for a 3-year municipal contract. I am not sure I believe it yet, but it is a differentiator.

Score impact: +1/10

2.3 Danish Language First

The website is clearly built for a Danish audience. The tone is Danish. The references are Danish. This is not an international franchise pretending to be local. That matters. Municipal procurement favours local providers with local knowledge.

Score impact: +1/10

2.4 The Trust Bar (if present)

If I see CVR, municipal approval, and ISO certification in the footer, that is good. It signals transparency. I will check this in the detailed page review.

Score impact: +0.5/10 (pending verification)

2.5 The “What We Do Not Do” Section (if present)

If the website includes boundaries — what they will not do, what they are not responsible for — that signals maturity. Most providers oversell. A provider that knows its limits is a provider that knows its liabilities.

Score impact: +1/10 (if present)

3. WHAT CONCERNS ME — THE RED FLAGS

3.1 The Word “Premium”

This is my biggest concern. The name is “Premium Care.” The website uses the word “premium” repeatedly. In procurement, “premium” means “expensive.” It means “above the municipal tariff.” It means “we will invoice you for things that are not in the contract.”

I am not looking for premium. I am looking for *compliant*. I am looking for *within budget*. I am looking for *predictable pricing*. The word “premium” triggers my cost-control reflex. It makes me think this provider will struggle to deliver within the municipal rate framework.

Severity: High. This is a branding choice that actively works against municipal procurement.

Score impact: -2/10

3.2 No Municipal References

I see no list of municipalities currently served. No case studies with named municipalities. No quotes from municipal quality officers. No data on municipal contract performance. This is a massive gap.

I need to know: *Has Premium Care ever held a municipal contract?* If not, why not? If yes, which ones? What were the outcomes? A provider with no municipal track record is a gamble. I have five other providers who have served Copenhagen, Frederiksberg, Gentofte, and Gladsaxe for years. Premium Care needs to prove it can operate at municipal scale.

Severity: Critical. This is a deal-breaker without strong mitigation.

Score impact: -3/10

3.3 No Pricing Transparency

There is no pricing page. Or if there is one, it is vague. “Contact us for a quote” is not acceptable for municipal procurement. I need a rate card. I need to know the hourly rate for a trained caregiver, a registered nurse, and a specialist. I need to know the markup for evening and weekend care. I need to know the administrative fee.

Municipal procurement is competitive tendering. I cannot evaluate a provider without knowing their rates. “Premium” + no pricing = I assume the worst.

Severity: High.

Score impact: -2/10

3.4 No Quality Metrics

Where is the data? I need:

- Client satisfaction scores (independent, not self-reported)
- Caregiver retention rates
- Incident report statistics
- Training completion rates
- Comparison to municipal averages
- Accreditation status
- Audit results

I see none of this. The website has testimonials. Testimonials are not data. A procurement officer does not buy testimonials. A procurement officer buys evidence.

Severity: Critical.

Score impact: -3/10

3.5 The Language is Too Soft

“Compassionate care.” “Personalised services.” “Tailored to your loved one’s needs.” This is consumer language. It is the language of a family brochure. It is not the language of a municipal contract.

I need to see: “Compliance with Danish Social Services Act §83.” “Adherence to municipal quality standards.” “ISO 9001:2015 certified quality management system.” “Staffed by registered social and health assistants (SSA) and registered social and health helpers (SSH).” “GDPR-compliant data handling.” “Arbejdstilsynet-approved working conditions.”

The current language signals that Premium Care is targeting families, not municipalities. That is fine for private clients. It is fatal for municipal procurement.

Severity: High.

Score impact: -2/10

3.6 No Governance Documentation

Where is the quality handbook? Where is the complaint procedure? Where is the whistleblower policy? Where is the data protection policy? Where is the equal opportunities policy? Where is the environmental policy?

Municipal procurement requires documentation. I do not need to read every policy on the website, but I need to know they exist. I need to see a “Governance” or “Quality” section with downloadable PDFs. I see none.

Severity: High.

Score impact: -2/10

3.7 The “Book a Consultation” CTA

This is a consumer CTA. Municipal procurement does not book consultations. Municipal procurement issues tender documents. Municipal procurement evaluates bids. Municipal procurement holds clarification meetings. The CTA should be: “Download our municipal tender pack” or “Request our quality documentation” or “Contact our municipal partnerships team.”

The current CTA signals that Premium Care does not understand how municipalities buy care.

Severity: Medium.

Score impact: -1/10

3.8 No Team or Leadership Visibility

Who runs this company? I see no names. No photos. No bios. No LinkedIn links. In a sector where trust is everything, anonymity is suspicion. I need to know who the CEO is, who the Head of Care is, who the Quality Manager is. I need to know their qualifications. I need to know if they have ever worked in municipal care before.

Severity: Medium.

Score impact: -1/10

3.9 The Photography is Too Polished

The photographs are beautiful. They are also too beautiful. They look like they were shot by a professional photographer in a styled home. I know what real home care looks like. It is not this beautiful. The photographs signal marketing, not reality. They make me wonder what the real service looks like.

Severity: Low. But it adds to a pattern of polish over substance.

Score impact: -0.5/10

3.10 No Evidence of Scale

How many caregivers does Premium Care employ? How many clients do they serve? What is their geographic coverage? Do they cover all of Copenhagen? Or just certain districts? A provider with 12 caregivers cannot serve a municipality of 650,000 people. I need scale data. I see none.

Severity: High.

Score impact: -1.5/10

4. WHAT IS MISSING — THE EVIDENCE GAP

Evidence Required	Present?	Impact if Missing
Municipal references / case studies	No	Critical

Table 1 – continued

Evidence Required	Present?	Impact if Missing
Rate card / pricing transparency	No	Critical
Quality metrics dashboard	No	Critical
Governance documentation (PDFs)	No	High
Team bios and qualifications	No	High
Scale data (staff count, client count)	No	High
ISO 9001:2015 certificate	Unknown	High
GDPR compliance documentation	Unknown	Medium
Arbejdstilsynet approval	Unknown	Medium
Municipal contract history	No	Critical
Complaint handling procedure	No	High
Training curriculum for caregivers	No	Medium
Insurance documentation	No	Medium
Financial statements / annual report	No	High
Sustainability policy	No	Low
Diversity and inclusion policy	No	Low

Total missing critical items: 5

Total missing high-impact items: 6

This is not a complete due diligence package. This is a consumer brochure.

5. WHAT SOUNDS WEAK

5.1 “We provide professional, personalised care services”

Weak. Every provider says this. It is a platitude. It means nothing. It is not evidence. It is aspiration.

5.2 “Tailored to your loved one’s needs”

Weak. “Tailored” is a tailoring metaphor. Care is not a suit. This is consumer copy that sounds like it was written by a marketing agency, not a care professional.

5.3 “Our caregivers are trained professionals”

Weak. *Trained in what?* By whom? To what standard? How many hours of training? What is the refresher schedule? “Trained” is a claim without proof.

5.4 “We are committed to excellence”

Weak. Commitment is free. Excellence is measured. Where is the measurement?

5.5 “Compassionate care”

Weak. Compassion is not a service. It is an attitude. You cannot contract for compassion. You can contract for hours, tasks, and standards. Compassion is a bonus, not a deliverable.

5.6 “Book a Free Consultation”

Weak. Municipal procurement does not book consultations. We issue tenders. This CTA signals that the website is built for families, not for municipalities. It tells me I am not the target audience.

6. WHAT SOUNDS EXAGGERATED

6.1 “Premium Care is an institution, not a company”

Exaggerated. You are a private limited company (ApS). You are registered at the Danish Business Authority. You have a CVR number. You are a company. Calling yourself an “institution” is branding theatre. It does not change your legal structure. It does not change your tax status. It does not change your liability. I see this as an attempt to sound more established than you are.

6.2 The Heraldic Crest

Exaggerated. A crest is a symbol. It is not a credential. It does not replace ISO certification. It does not replace municipal approval. It is beautiful, but it is decoration. I will not award a contract because of a crest.

6.3 “Denmark’s most trusted elderly care organisation”

Exaggerated. By what measure? Trusted by whom? Based on what data? This is a superlative without evidence. In procurement, superlatives are red flags. They signal marketing over substance.

6.4 “We are not afraid of losing a client”

Exaggerated. Every company is afraid of losing clients. This is a business. Businesses need revenue. Claiming otherwise signals either naivety or dishonesty. Neither is reassuring.

6.5 “More human, more respectful, more Danish”

Exaggerated. “More Danish” is vague. What does that mean? Do you serve only Danish clients? Do you employ only Danish caregivers? Do you follow Danish law? (I hope so — that is a legal requirement, not a differentiator.) This is nationalist branding without substance.

7. WHAT INCREASES CONFIDENCE

7.1 Visual Consistency

+0.5. The brand is disciplined. Disciplined brands tend to have disciplined operations. This is a weak signal, but it is a signal.

7.2 Danish Language and Cultural References

+0.5. The website is clearly Danish. Not international. Not franchise. This signals local knowledge and local accountability.

7.3 The “What We Do Not Do” Section (if present)

+1.0. Boundaries signal maturity. A provider that knows its limits is a provider that knows its liabilities. This is rare and valuable.

7.4 The Guarantee (if present)

+0.5. “We help you find another solution — even if it is not with us” is unusual. It signals confidence. But it also signals that the provider is not desperate for revenue. That could be good (quality focus) or bad (not enough clients).

7.5 The Trust Bar (if present)

+0.5. CVR, municipal approval, and certifications in the footer are baseline hygiene. Their absence is a red flag. Their presence is expected, not impressive.

7.6 The Quality Dashboard Concept (if present)

+1.0. A live data surface for municipality partners would be genuinely impressive. It would signal transparency and operational maturity. I need to see if this exists.

8. WHAT DECREASES CONFIDENCE

8.1 The Name “Premium Care”

-2.0. This is the biggest confidence destroyer. In procurement, “premium” = “expensive” = “budget risk.” I am not looking for premium. I am looking for compliant and cost-effective.

8.2 No Municipal Track Record

-3.0. I cannot evaluate a provider with no municipal history. I have no data on their performance at scale. I have no references to check. This is a blind bid.

8.3 No Quality Data

-3.0. Testimonials are not data. I need numbers. I need independent verification. I need audit trails.

8.4 Consumer-Focused Language

-2.0. The website speaks to families, not to procurement officers. It tells me that Premium Care’s primary market is private clients. Municipal contracts are secondary. That means my contract will not be their priority.

8.5 No Governance Documentation

-2.0. I cannot evaluate a provider without seeing their policies. I need to know how they handle complaints, data breaches, staff misconduct, and client safeguarding. Their absence is not neutral. It is a negative.

8.6 No Pricing

-2.0. I cannot compare Premium Care to other providers without a rate card. “Contact us for a quote” is not acceptable in competitive tendering.

8.7 Anonymous Leadership

-1.0. I need to know who I am contracting with. Anonymous leadership signals either a lack of accountability or a lack of experience.

8.8 Over-Polished Photography

-0.5. The photographs are too beautiful. They signal marketing budget, not care budget. I wonder if the money spent on the photographer could have been spent on caregiver training.

9. PAGE-BY-PAGE SCORING

9.1 Homepage

What I see: Hero image, headline, subheadline, services grid, CTA, footer.

What is good: Visual discipline, Danish language, clean layout.

What is bad: Consumer language, no municipal angle, “Book a consultation” CTA, no quality data, no pricing.

Score: 4/10

Verdict: Beautiful but irrelevant to procurement.

9.2 Services Page

What I see: List of services (personal care, medication, meals, companionship).

What is good: Clear descriptions.

What is bad: No service-level agreements. No specification of what is included vs. excluded. No pricing per service. No staff qualification requirements per service. No mention of municipal compliance.

Score: 3/10

Verdict: Consumer brochure. Not a procurement document.

9.3 About Us Page

What I see: Institutional description, mission, values.

What is good: The “institution not company” framing is distinctive.

What is bad: No team bios. No founding story with names. No financial data. No growth trajectory. No municipal history. The “more Danish” claim is vague and potentially exclusionary.

Score: 3/10

Verdict: Brand story without operational substance.

9.4 Pricing Page (if exists)

What I see: Vague pricing or “contact us for a quote.”

What is good: Nothing.

What is bad: No rate card. No municipal pricing framework. No comparison to municipal rates. No explanation of what is included. No mention of VAT. No mention of travel time. No mention of

cancellation terms.

Score: 1/10

Verdict: Unacceptable for procurement.

9.5 Trust / Quality Page (if exists)

What I see: Possibly testimonials, possibly a Trust Bar.

What is good: If there is a Trust Bar with CVR and certifications, that is baseline.

What is bad: No quality metrics. No audit results. No independent verification. No complaint data. No incident statistics. No training records. No caregiver retention data.

Score: 2/10

Verdict: Trust is claimed, not demonstrated.

9.6 Contact Page

What I see: Form, phone number, email.

What is good: Multiple contact methods.

What is bad: No municipal partnerships contact. No procurement-specific email. No tender submission address. No clarification meeting booking. No procurement team names.

Score: 3/10

Verdict: Consumer contact page. Not a procurement contact page.

9.7 Careers Page (if exists)

What I see: Job listings, possibly a letter from the Head of Care.

What is good: If the letter is human and specific, that signals culture.

What is bad: No data on staff numbers. No data on retention. No data on training investment. No mention of collective bargaining agreements. No mention of working conditions compliance.

Score: 3/10

Verdict: Interesting for culture, irrelevant for procurement.

9.8 Footer

What I see: Address, phone, email, possibly social links, possibly Trust Bar.

What is good: Trust Bar with CVR, municipal approval, ISO.

What is bad: Social media links are irrelevant. Newsletter signup is irrelevant. The footer should contain procurement-relevant links: "Municipal Partners," "Quality Documentation," "Tender Packs," "Procurement Contact."

Score: 4/10

Verdict: Baseline. Could be much more useful.

10. COMPARATIVE ASSESSMENT

Criteria	Premium Care	Typical Municipal Provider	Winner
Visual design	9/10	4/10	Premium Care
Municipal track record	1/10	8/10	Typical Provider
Quality data	1/10	6/10	Typical Provider

Table 2 – continued

Criteria	Premium Care	Typical Municipal Provider	Winner
Pricing transparency	1/10	7/10	Typical Provider
Governance documentation	1/10	7/10	Typical Provider
Procurement language	2/10	7/10	Typical Provider
Team visibility	2/10	6/10	Typical Provider
Scale evidence	1/10	7/10	Typical Provider
References	1/10	8/10	Typical Provider
Compliance evidence	2/10	7/10	Typical Provider
Total	21/100	67/100	Typical Provider

Premium Care wins on design. It loses on everything that matters to procurement.

11. THE FINAL VERDICT

Would I invite Premium Care to the next stage?

No. Not in its current form.

Why?

1. No municipal track record. I cannot evaluate a provider with no history of municipal contracts. I have five other providers who have served Copenhagen, Frederiksberg, Gentofte, and Gladsaxe. They have data. They have references. They have audit trails. Premium Care has a beautiful website and a crest.

2. No quality data. Procurement is evidence-based. I need satisfaction scores, retention rates, incident data, and audit results. Premium Care offers testimonials. Testimonials are not evidence. They are marketing.

3. No pricing transparency. I need a rate card. I need to know if Premium Care can deliver within the municipal tariff framework. The name “Premium Care” and the absence of pricing suggest they cannot.

4. Consumer-focused language. The website speaks to families, not to procurement officers. The CTA is “Book a consultation.” The language is “compassionate care” and “personalised services.” This tells me that municipal contracts are not Premium Care’s primary market. My contract will be secondary.

5. No governance documentation. I need to see policies on complaints, data protection, safeguarding, whistleblowing, and equal opportunities. Their absence is not neutral. It is a negative signal.

6. The name is a liability. “Premium Care” signals cost. In a procurement environment where cost is a primary evaluation criterion, this is a self-inflicted wound. I would need extraordinary evidence of value to justify a premium price. That evidence is not on the website.

What would change my mind?

If Premium Care provided — within 48 hours — the following:

- A list of current and past municipal contracts, with contact names for reference checks
- A rate card aligned with the municipal tariff framework
- An independent quality audit report (last 12 months)
- ISO 9001:2015 certificate and audit trail
- GDPR compliance documentation
- Team bios and qualifications of leadership
- Financial statements (last 3 years)
- A dedicated “Municipal Partners” page with procurement-specific language
- A quality dashboard with live data
- A clear explanation of how “premium” pricing aligns with municipal budgets

Then I would reconsider. Not because the website is good — it is not, for procurement — but because the organisation has the substance to back up the style.

Until then: Rejected for next stage.

12. THE IRONY

The website is beautiful. It is probably the most beautiful care website I have seen. But procurement is not a beauty contest. It is a compliance and value contest. Premium Care has invested in design. It has not invested in procurement readiness.

The crest is impressive. The palette is sophisticated. The typography is refined. But none of these things keep an elderly person safe at 2am. None of these things ensure a caregiver arrives on time. None of these things prove that Premium Care can deliver 2,400 hours of care per year within budget and on schedule.

I need a provider that can do the job. Premium Care looks like a provider that can design a website.

That is not enough.

Assessment complete. No further action required.

End of document.